Nivolumab + ipilimumab · colorectal after chemotherapy

Colorectal treatment setting MSI-H/dMMR metastatic colorectal cancer after fluoropyrimidine-based combination chemotherapy. Other nivolumab/ipilimumab regimens have different doses, routes and timing.

Supportive prescription draft for review. This is a regimen template. It is not an issued patient prescription; individual review and completion are required.

Cycle 5 onwards: prescribed nivolumab monotherapy schedule

Active medicines: Nivolumab

Emetogenic risk: Minimal

No routine prophylactic antiemetic order is populated for this phase. Agree an individual symptom-treatment plan where needed.

Additional phase-specific support

- No routine antiemetic prophylaxis or infusion premedication; prescribe symptom relief individually.
- Carry the immunotherapy alert card and report bowel, lung, liver, kidney, hormonal, skin, neurological or cardiac symptoms promptly during and after treatment.
- full blood count (FBC), renal/liver tests, glucose and thyroid function before each cycle; cortisol as clinically indicated. Maintain follow-up for delayed immune toxicity.
- Management uses dose delay or discontinuation, not routine dose reduction. High-dose steroids for immune toxicity require a separately prescribed treatment and taper plan.
- Cycles 1–4: nivolumab 3 mg/kg, then ipilimumab 1 mg/kg, on Day 1 every 21 days. The first nivolumab-only dose is due three weeks after the final combination dose, subject to clinical review. Nivolumab alone: 240 mg intravenous (IV) or 600 mg under the skin every two weeks, or 1200 mg under the skin every four weeks.
- If the team changes the interval between nivolumab-only doses, they will give you a new appointment plan: the first four-weekly dose is two weeks after the last two-weekly dose, or the first two-weekly dose is four weeks after the last four-weekly dose. Do not change the schedule yourself. This does not change which formulations are permitted for your colorectal treatment.

Before signing: verify allergies, current medicines, interactions, renal / hepatic function, QT risk, sedation, prior response, total steroid exposure and the rescue-medicine duration. Do not duplicate medicines from another phase.

Individual changes / rescue duration: _____
Prescriber / date: _____ Pharmacy review: _____

NCCP 00551 v7a · p2,10 · NCCP regimen; NCIS Medical Oncology antiemetics V8 Table 1

Sources checked 2026-11-05 · SACTCheck v0.84.35 · Regimen-level drafts; prescriber and oncology-pharmacy verification required